

Neuropsychological assessment

Summary of the results

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Assessment carried out in November 2024 by a clinical psychologist of the team of [Dr Stefano Bramante](#), psychiatrist, the referring specialist.

Cognitive profile: WAIS-IV

Wechsler Adult Intelligence Scale, fourth edition (Italian edition, 2013): composite scores have mean 100 and standard deviation 15.

scale	sum of scaled scores	composite score	percentile	95% confidence interval
Verbal Comprehension (VCI)	40	123	94	116–128
Perceptual Reasoning (PRI)	37	113	81	105–119
Working Memory (WMI)	22	103	58	95–111
Processing Speed (PSI)	22	106	66	98–114
Full Scale IQ (FSIQ)	121	117	87	111–122

The profile is uneven: the differences VCI–WMI and VCI–PSI are significant. Verbal comprehension, in the superior range, is the main strength, and perceptual reasoning is also above the indices of cognitive efficiency (working memory and processing speed).

ADHD: DIVA-5

Diagnostic Interview for ADHD in adults, fifth edition: number of criteria met.

	cut-off	criteria met	outcome
childhood, inattention	>3	4	present
childhood, hyperactivity/impulsivity	>3	1	within norm
adulthood, inattention	>5	7 *	present
adulthood, hyperactivity/impulsivity	>5	1	within norm

ADHD: BAARS-IV

Barkley Adult ADHD Rating Scale, fourth edition, filled in by me (self) and by a family member (other), for childhood and for the present.

	self, childhood	self, present	other, childhood	other, present
inattention	27 (98 *)	33 (99 *)	28 (98 *)	35 (99 *)
hyperactivity/impulsivity	13 (51–75)	—	17 (88)	—
impulsivity	—	6 (51–75)	—	14 (99 *)
hyperactivity	—	5 (1–50)	—	8 (81)
sluggish cognitive tempo	—	33 (99 *)	—	26 (97 *)

Raw score, with its percentile in brackets; in bold and with an asterisk the scores in the clinical range.

Conclusion of the report

The overall intellectual profile is high average (FSIQ 117), with a significant internal unevenness: strengths in verbal and perceptual reasoning, and a relative weakness in working memory and processing speed, together with attention and executive difficulties present since early childhood. The interviews and the scales, both self-reported and reported by a family member, are positive for childhood and for adulthood, and the symptoms of adulthood are persistent and not secondary to other conditions. The profile is compatible with ADHD, predominantly inattentive presentation.